



PATIENT HANDOFF NOTE

■■ CRITICAL PATIENT INFORMATION ■■

Patient Name:	Henry Cavill
Patient ID:	49
Handoff Date:	October 17, 2025 10:52
Shift From:	Day Shift
Shift To:	Night Shift
Created By:	Staff ID: 31

PATIENT OVERVIEW

Henry Cavill is a patient with unknown age and gender. No significant past medical history is available. Based on recent appointment history, the patient is currently presenting with recurrent episodes of "hurting" and has a documented appointment reason of "cancer" on 2025-10-20, suggesting a new diagnosis or workup for malignancy and associated symptoms.

CURRENT CONDITION

Patient's current clinical status and stability are unknown due to a complete lack of recent vital signs, lab results, and documented clinical assessment. Patient is presumed symptomatic given the repeated complaints of "hurting." The potential diagnosis of cancer represents a significant clinical concern requiring immediate

investigation.

ACTIVE PROBLEMS

* Undifferentiated Pain/Hurting: Documented repeatedly (2025-10-17, 2025-10-21, 2025-10-20). Requires thorough assessment of onset, character, location, radiation, aggravating/alleviating factors, and severity. * Suspected/Diagnosed Malignancy: Documented as an appointment reason ("cancer" on 2025-10-20). Requires urgent diagnostic workup, staging, and oncology consultation.

RECENT CHANGES (24-48 HRS)

Over the last 24-48 hours (based on appointment dates): * Patient had an appointment regarding "cancer" on 2025-10-20, which is the most significant new development. * Continued complaints of "hurting" are noted on 2025-10-20 and 2025-10-21, indicating persistent or recurrent symptoms.

CURRENT MEDICATIONS

None documented.

PENDING TASKS

* Comprehensive History & Physical: Obtain detailed history from patient (if able), including age, gender, social history, and complete review of systems. Perform a thorough physical examination. * Vital Signs: Obtain a full set of vital signs immediately and continue to monitor per protocol. * Pain Assessment & Management: Conduct a detailed pain assessment and initiate appropriate analgesic regimen as indicated. * Diagnostic Workup for Malignancy: * Review previous records related to the "cancer" appointment. * Order initial laboratory studies (e.g., CBC, CMP, LDH, tumor markers if clinically indicated). * Order urgent imaging (e.g., CT chest/abdomen/pelvis, MRI, PET scan) to define extent of disease. * Consult with Oncology for further management, biopsy, and staging. * Basic Labs: Order routine admission labs (e.g., CBC, BMP, LFTs, UA). * Consultations: Consider Palliative Care for symptom management and support, given the potential cancer diagnosis and pain. * Documentation Review: Investigate the "adasd" appointment reason for any missed information.

■■■ IMPORTANT ALERTS

* Allergies: None documented. Confirm with patient. * Code Status: Not documented. Discuss with patient and family once present and assess. * Critical Information Gap: Significant lack of demographic and clinical data (age, gender, PMH, vitals, labs, medications) severely impedes initial assessment and risk stratification. Requires immediate attention to fill these gaps. * Safety Precaution: Close monitoring for acute changes in condition, especially related to pain or potential oncologic emergencies, is paramount.

CARE PLAN

Goals for next shift: 1. Stabilization & Symptom Control: Assess and manage current symptoms, particularly pain. 2. Diagnostic Clarification: Initiate urgent workup for suspected malignancy. 3. Data Acquisition: Obtain all missing demographic and clinical information. Ongoing Plan: * Complete comprehensive H&P; vital signs, and

initial labs. * Aggressively manage pain as per assessment. * Expedite diagnostic imaging and pathology for cancer workup. * Establish oncology consultation. * Monitor closely for changes in clinical status, signs of instability, or oncologic complications.

HANDOFF ACKNOWLEDGMENT

Receiving Staff Signature:

Date/Time:

Printed Name:
